CASE SCENARIO: I USED FOR ASSESSING CAUSALITY IS

CASE SCENARIO:

Patient Name: PHCG

Age: 92 Year

Gender: Male

Allergy Status: No Known Allergy

Past Medical History: Alzheimer’ s Disease, Old IHD, BPH

Present Illness: Acute CVA – GC bleed extending to right corona radiata with left hemiparesis, Sepsis – Aspiration Pneumonia (Resolving).

Past Medication History:

T. ESCOSPRIN AV (75/10MG) 0-0-1
T. VELTAM S 0-0-1
T. DAPAGLIFLOZIN 5MG 0-1-0
T CIDMUS 50MG 1/2-0-0
T. CARDIVAS CR 10MG 1-0-0
T. OXRA 5 MG 1-0-0

On 24/09/2026 The Patient Serum Potassium Level Declined to **3.2 mmol/L**

On 25/09/2026 The Patient Serum Potassium Level Declined to **3.2 mmol/L**

On 26/09/2026 The Patient Serum Potassium Level Declined to **3.0 mmol/L**

LABORATORY DATA:

Physical Examination:

Blood Pressure: 110/70 mmHg

Heart Rate: 120 BPM

Respiratory Rate: 22CPM

MANAGEMENT:

On 24/09/2026 She was started with Syp Potklor 10ml TID

On 25/09/2026 She was treated with Inj KCl 2 AMP in 500ml fluids @ 100CC/hrs

The Decreased Serum Potassium Level (Hypokalemia) was resolved on 27/09/2026

CAUSALITY ASSESSMENT USING NARANJO SCALE AND WHO-UMC SCALE OF CASE SCENARIO WHICH I HAVE SELECTED:

2. CAUSALITY ASSESSMENT

Naranjo Adverse Drug Reaction Probability

Question	Yes	No	Do Not Know	Score
1. Are there previous *conclusive* reports on this reaction?	+1	0	0	+1
2. Did the adverse event appear after the suspected drug was administered?	+2	-1	0	+2
3. Did the adverse reaction improve when the drug was discontinued or a *specific* antagonist was administered?	+1	0	0	+1
4. Did the adverse event reappear when the drug was re-administered?	+2	-1	0	0
5. Are there alternative causes (other than the drug) that could on their own have caused the reaction?	-1	+2	0	0
6. Did the reaction reappear when a placebo was given?	-1	+1	0	0
7. Was the drug detected in blood (or other fluids) in concentrations known to be toxic?	+1	0	0	0
8. Was the reaction more severe when the dose was increased or less severe when the dose was decreased?	+1	0	0	0
9. Did the patient have a similar reaction to the same or similar drugs in *any* previous exposure?	+1	0	0	0
10. Was the adverse event confirmed by any objective evidence?	+1	0	0	+1
		TOTAL SCORE:		05

SCORING FOR NARANJO ALGORITHM > 9 = DEFINITE ADR 5-8 PROBABLE ADR 1-4 = POSSIBLE ADR 0= DOUBTFUL ADR

WHO – UMC causality categories

Causality term	Assessment criteria
Certain	• Event or laboratory test abnormality, with plausible time relationship to drug intake • Cannot be explained by disease or other drugs • Response to withdrawal plausible (pharmacologically, pathologically) • Event definitive pharmacologically or phenomenologically (i.e. an objective and specific medical disorder or a recognised pharmacological phenomenon) • Rechallenge satisfactory, if necessary
Probable / Likely	• Event or laboratory test abnormality, with reasonable time relationship to drug intake • Unlikely to be attributed to disease or other drugs • Response to withdrawal clinically reasonable • Rechallenge not required
Possible	• Event or laboratory test abnormality, with reasonable time relationship to drug intake • Could also be explained by disease or other drugs • Information on drug withdrawal may be lacking or unclear
Unlikely	• Event or laboratory test abnormality, with a time to drug intake that makes a relationship improbable (but not impossible) • Disease or other drugs provide plausible explanations
Conditional/ unclassified	• Event or laboratory test abnormality • More data for proper assessment needed, or • Additional data under examination
Unassessable / Unclassifiable	• Report suggesting an adverse reaction • Cannot be judged because information is insufficient or contradictory • Data cannot be supplemented or verified

CAUSALITY ASSESSMENT USING NARANJO SCALE AND WHO-UMC SCALE OF CASE SCENARIO OF THE TUTORIAL VEDIO:

2. CAUSALITY ASSESSMENT

Naranjo Adverse Drug Reaction Probability

Question	Yes	No	Do Not Know	Score
1. Are there previous *conclusive* reports on this reaction?	~~+1~~	0	0	+1
2. Did the adverse event appear after the suspected drug was administered?	~~+2~~	-1	0	+2
3. Did the adverse reaction improve when the drug was discontinued or a *specific* antagonist was administered?	~~+1~~	0	0	+1
4. Did the adverse event reappear when the drug was re-administered?	+2	-1	~~0~~	0
5. Are there alternative causes (other than the drug) that could on their own have caused the reaction?	-1	~~+2~~	0	+2
6. Did the reaction reappear when a placebo was given?	-1	+1	~~0~~	0
7. Was the drug detected in blood (or other fluids) in concentrations known to be toxic?	+1	~~0~~	0	0
8. Was the reaction more severe when the dose was increased or less severe when the dose was decreased?	+1	0	~~0~~	0
9. Did the patient have a similar reaction to the same or similar drugs in *any* previous exposure?	+1	0	~~0~~	0
10. Was the adverse event confirmed by any objective evidence?	+1	~~0~~	0	0
			TOTAL SCORE:	+6

SCORING FOR NARANJO ALGORITHM > 9 = DEFINITE ADR 5-8 PROBABLE ADR 1-4 = POSSIBLE ADR 0= DOUBTFUL ADR

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